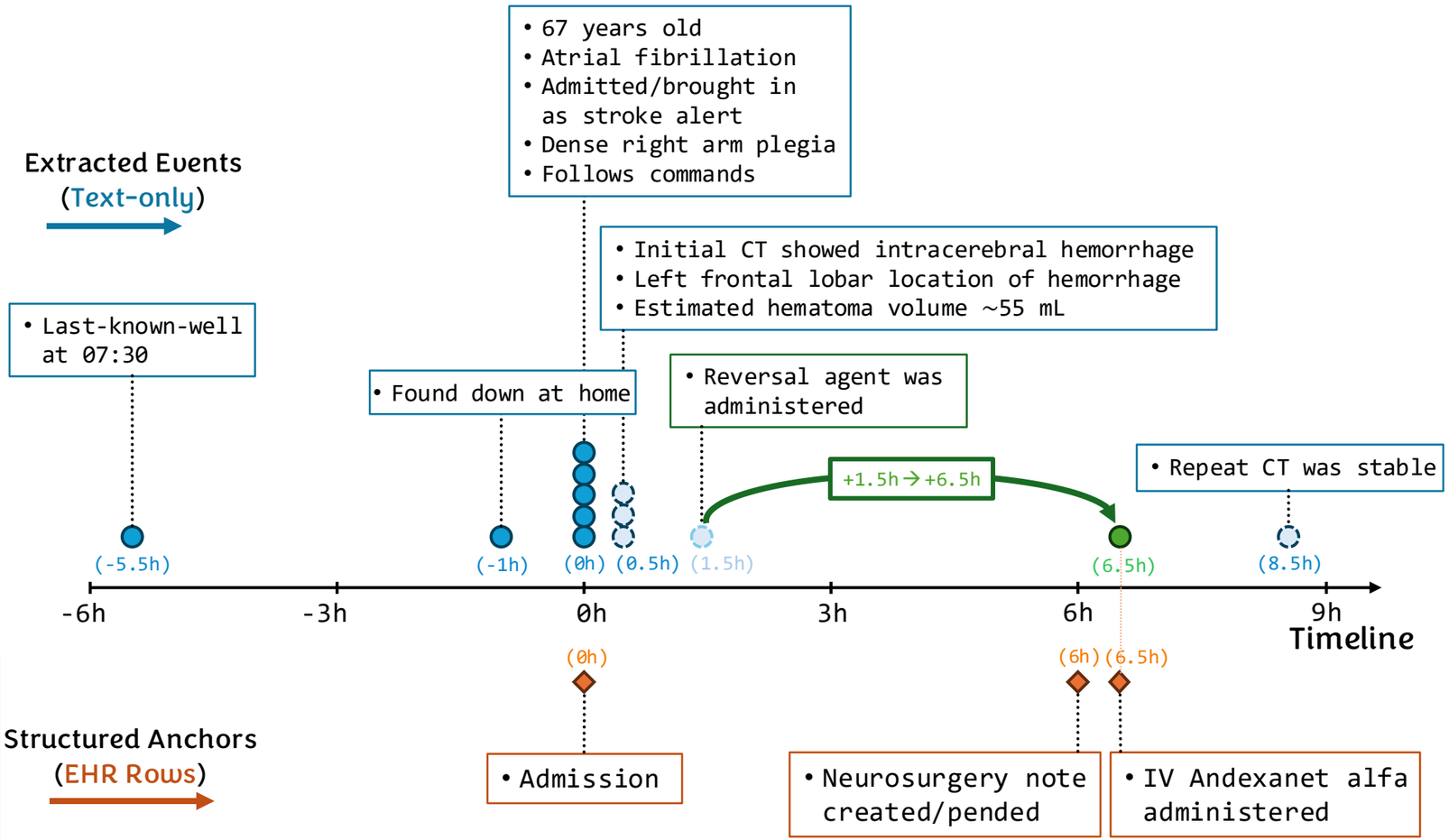
(A) Text-only Reconstruction: Rich phenotype, ambiguous intervention time

Clinical Note (T)

A 67-year-old man with atrial fibrillation on a direct-acting oral anticoagulant is found down at home and brought in as a stroke alert. Last-known-well time is 0730. He follows commands but is non-verbal/aphasic with dense right arm plegia. Initial CT shows left frontal lobar intracerebral hemorrhage with estimated hematoma volume ~55 mL, with no intraventricular hemorrhage or hydrocephalus. Reversal agent was administered, and repeat CT is stable.

Structured EHR Rows (R)

Event	Value	Timestamp
Admission	——	04/14/2026 13:00
neurosurgery note created /pended	Speech deficit	04/14/2026 19:00
...		
IV Andexanet alfa administered	880mg	04/14/2026 19:30
...		



(B) Retrieval-augmented Alignment: Selective tabular correction

Narrative text preserves:

- Last-known-well and presentation context
- Aphasia and dense right arm plegia
- Hemorrhage location and volume
- Repeat CT stability

Structured rows calibrate:

- Admission reference time
- Precise reversal agent timing via IV Andexanet alfa

Result:

- Clinical phenotype remains narrative derived
- Reversal agent administration shifts from +1.5h to +6.5h

- Text-derived event time
- Text-only estimate
- Structured EHR anchor
- Calibrated event time